DPP DEPARTMENTAL POLICY AND PROCEDURE

TITLE:	DISCHARGE CRITERIA
DEPARTMENT:	INTENSIVE CARE UNIT
APPLIES TO:	ALL MEDICAL AND NURSING STAFF

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1. PURPOSE

1.1. To set procedural guidelines on how to discharge patient in intensive care unit.

1.2. To determine patient who need ICU care according to priorities in accordance to ICU bed capacity.

1.3. To avoid re-admission after transferring unstable patient to the ward.

1.4. To set regulation on when to discharge the patient to the ward.

2. DEFINITION & ABBREVIATION

2.1. **ICU** – Intensive Care Unit

2.2. **NOD** – Nurse on Duty

2.3. **Designee** – is a person selected or designated to carry out a duty or role.

2.4. **HIS** – Hospital Information System (MCC – Medica Cloud Care System)

3. RESPONSIBILITY

3.1. **Assistant Executive Director for Medical Services** - ensures that all departments adhere to this policy.

3.2. **Head of Adult ICU**

3.2.1. Overall in-charge of ICU

3.2.2. Ensures that patient meet eligibility requirement to discharge from ICU and discharge stable patient regularly and as early as possible to provide bed for more acute and new patients who needs intensive care.

3.3. **ICU Physician** – assess patient and discuss patient's condition with the Chief of ICU and/or attending physician or designee for stable patient that conforms to discharge criteria and can be manage in the unit/ward.

3.4. **Attending physician and Medical Specialist on-Call/Designee** - discuss patient's condition with ICU Physician if suitable to manage in the ward and take full responsibility when patient was shifted to the ward.

3.5. **ICU Head Nurse** – ensures that all staff is aware of this policy and is being followed in the department.

3.6. **ICU Nurse** – informs the ward nurse about the condition of the patient, the equipment and/or machines needed by the patient in the ward.

3.7. **Unit/ Ward Nurse** – prepares necessary equipment and/or machines needed by the patient prior to receiving the patient in the unit.

4. POLICY

4.1. Discharge from ICU must be according to criteria and priorities:

4.1.1. When the need for intensive treatments no longer present, or when treatment has failed and short-term prognosis is poor.

4.1.2. When likelihood of suddenly requiring intensive treatment has diminished.

4.1.3. When the need for intensive treatment is no longer present, but may be discharged earlier if there is little likelihood or recovery or benefit from continued invasive treatment.

4.1.4. **Medical Patients:**

4.1.4.1. A medical patient can be discharged from the ICU if his/her physical status has stabilized and the need for ICU monitoring and care is no longer necessary, i.e., the pathological process necessitating admission to the ICU has resolved or significantly improved. For patient discharge, the following criteria are to fulfilled:

4.1.4.2. Hemodynamic ally stable for ≥12 hours off Inotropic Support

4.1.4.3. Oxygen requirement <40% FiO2 with oxygen Sat ≥92% and acceptable ph.

4.1.4.4. Extubated for >6 hours with no significant upper airway problems i.e., good cough and signs of respiratory distress.

4.1.4.5. Has no excessive fluid loss or requirements.

4.1.4.6. For patients who continue to deteriorate and in whom active intervention is deemed futile, discharge to a low level of care, i.e. special care unit is appropriate.

4.1.5. **Surgical Patients:**

4.1.5.1. Hemodynamic ally stable for ≥12 hours, off Inotropic Support and no significant bleeding

4.1.5.2. Oxygen requirement <40% FiO2 with oxygen Sat ≥92% and acceptable ph.

4.1.5.3. Extubated for ≥6 hours with no significant upper airway problems i.e., good cough and signs of respiratory distress.

4.1.5.4. For patients who continue to deteriorate and in whom active intervention is deem futile, discharge to a lower level of care, special care unit or floor is appropriate.

5. PROCEDURE

5.1. The patient will be evaluated by the Head of ICU department or his designee during morning rounds and transcribes his plan to shift to the ward when patient meet discharge criteria.

5.2. The attending physician will be informed by the ICU physician during his regular visit to the patient.

5.3. When patient conforms to discharge criteria, the attending physician and the ICU physician will inscribe his order and the treatment to be continued in the ward in the MCC.

5.4. In case of emergency that demand for beds exceeds availability, the Head of ICU department or his designee determines which patient is to be given priority.

5.5. When the attending physician disagrees to shift patient to the ward, he should discuss the reason to the ICU physician and indicate it in the progress notes.

5.6. Medical director might set a committee to retrospectively review cases wherein the attending physician has disagreed with the decision of the ICU Consultant.

5.7. Transfer patient to the ward. (Refer to DPP-ICU-0020: Transfer of Patient from ICU to Ward).

6. MEASUREMENT

6.1. 100% of the medical and nursing staff are well oriented and adheres to this policy.

7. REFERENCES /CROSS- REFERENCES

7.1. King Fahad Hospital Policy and Procedure: ICU discharge

8. ATTACHMENT

8.1. ICU Nurses Internal Transfer / Discharge Form during downtime

8.2. Transfer Summary Report (Critical Unit to Lower Acuity)

9. REVISION CLAUSE

9.1. Changes to the hospital policy and procedure template based from the Al-Baha Health Cluster logo, font face and design

9.2. Modification to responsibility contents.

AL-Baha Health Cluster
تجمع الباحة الصحي

PRINCE MISHARI BIN SAUD HOSPITAL IN BALJURASHI
مستشفى الأمير مشاري بن سعود ببلجرشي

MRN: | | | | | | | | |

Name: _____

Age: | | | | Yrs/Months: Sex M: | | F: | |

Nationality _____

Consultant In charge: _____

Dept: _____ Unit: _____

TRANSFER SUMMARY REPORT (Critical to Lower Unit)

Date of Admission:			Date of Transfer to Lower Acuity:		
Transferring Department:			Receiving Department:		

Brief Medical History:

Physical Examination:

Laboratory Summary Report:

Radiology Summary Report:

Radiology Summary Report:

Medical Management (Procedures/Treatment Received):

Status upon Transfer to Lower Acuity:

☐ Cured ☐ Improved ☐ Not Improved ☐ Others/Specify: _____

Endorsing Physician: _____ Signature: _____ Date/Time: _____

Receiving Physician: Signature: Date/Time: